

CQC 28-Day Strategic Roadmap

For Dr Omer Atli

Focused on mitigation, registration logistics, premises/location risk, home-visit structure, and the Louise pathway

Prepared 8 May 2026 | Based on current CQC, [legislation.gov.uk](https://www.legislation.gov.uk) and related official guidance reviewed on 8 May 2026

Important: This document is a strategic working paper, not formal legal advice. It is designed to help you brief your solicitor, organise evidence, and avoid making avoidable mistakes in the 28-day response window. Do not send any substantive representation to CQC without your lawyer reviewing it first.

Executive summary

Your strongest position is not a clean merits defence that CQC definitely had no jurisdiction. Your strongest position is that the territorial scope was genuinely confusing, you misunderstood how CQC applies the “carried on in England” concept to online care, and you stopped immediately once contacted.

That argument has real weight because CQC’s own materials pull in different directions. One current page says CQC regulates online primary care services in England. Another says if you are based outside England but are carrying on regulated activity in England, you will be expected to supply an address in England. But CQC’s consumer page also says it can only register services based in England, and the 2022 scope guidance says CQC does not normally regulate providers based outside England that offer care to patients in England.

Those tensions do not guarantee you win. They do, however, materially strengthen a good-faith misunderstanding narrative and weaken any suggestion that you were deliberately thumbing your nose at the regulator.

If you now want to apply as an individual provider, the public route is form-based, document-heavy, and not a casual click-through process. There is no initial registration fee, but there will be annual provider fees once registered. An enhanced DBS is mandatory. CQC can reject incomplete applications. For online primary care you also need the extra online primary care form and service-specific supporting documents.

A Turkey-only premises strategy looks weak. Current CQC location guidance says that if you are based outside England but carrying on regulated activity in England, you will be expected to supply an address in England. It also says a virtual office or PO box will not count as a location. So a Turkey-only address is not a clean published route, and a fake or soft UK address is a terrible idea.

Home visits are not a loophole. CQC’s inspection guidance expressly includes private GP services and medical agencies that visit people in their home or hotels/care homes. The medical practitioner exemption does not apply to treatment outside a surgery or consulting room, so home visits do not rescue an otherwise registrable model.

For Louise, the commercially meaningful message is this: CQC is not just a burden badge. It is the structure that lets a clinic become a serious healthcare platform rather than an informal arrangement. But the provider question has to be designed properly. If you remain the legal person directing and controlling the regulated activity, using Louise’s premises alone may not solve your registration problem.

1. What the official sources really say, and why that helps your mitigation narrative

1.1 The legal hook

The legislation says: “An activity is only a regulated activity if it is carried on in England.” That is regulation 3(3) of the 2014 Regulated Activities Regulations. On paper, that wording is the seed of your misunderstanding argument.

But paper wording is not the whole game. CQC looks at how the service is actually carried on, who is directing and controlling it, what the patient sees, where the service is managed from, and what kind of online medical care is being delivered.

1.2 The official tension inside CQC’s own guidance

Current online-primary-care guidance says CQC regulates providers of online primary care services in England where they are delivering a regulated activity by online means, including video consultations and prescribing in response to online forms.

Current location guidance says if a provider is based outside England but is carrying on regulated activity in England, that provider will be expected to supply an address in England.

But CQC’s consumer-facing guidance also says it can only register services that are based in England.

And the still-published 2022 scope guidance says CQC does not normally regulate providers based outside England that offer care and treatment to patients in England.

Why this matters strategically:

It does not create a comfortable “I was obviously exempt” argument.

It does create a credible “I misunderstood a genuinely messy territorial rule-set” argument.

That is the lane to stay in. If you try to become a crusader for the proposition that CQC definitely had zero reach, you risk sounding evasive. If you frame it as a good-faith misreading corrected immediately when clarified, you sound cooperative and remediable.

2. The safest defence framing for the next 28 days

Your proposed framing is broadly sound, but it needs discipline. The point is not: “I was in Turkey, therefore no offence.” The point is: “Because I was operating from Turkey, without a UK clinical premises, I genuinely understood the position to be outside CQC registration scope. Once CQC clarified its view, I immediately ceased the activity and began taking advice.”

This is stronger because it accepts CQC’s seriousness, avoids open combat, and foregrounds immediate remediation.

You should not overclaim. Do not say “the day you emailed me I permanently shut every possible pathway forever” unless you can evidence every single step. Instead say what you actually did and can prove.

You also should not quietly slip into Option 3 language while pretending to choose Option 2. If the strategy is mitigation-first, keep the focus on cessation, misunderstanding, cooperation, chronology, and preserved evidence. Save any nuanced territorial arguments for lawyer-shaped wording, not chest-thumping prose.

2.1 Points worth preserving for a lawyer-drafted response

- You were physically based in Turkey when carrying out the activity.
- You did not understand CQC to require registration in these cross-border circumstances.
- CQC guidance itself contains different formulations about England-based services, outside-England providers, and carrying on activity in England.
- On receipt of the enforcement letter, you immediately ceased the activity and began reviewing the position.
- You are taking legal advice and considering compliant future structures rather than continuing in defiance.
- There has been no deliberate concealment and no continuation after notice.

2.2 Points to avoid

- Do not call yourself definitely exempt.
- Do not say CQC has no jurisdiction over you full stop.
- Do not rely on old pandemic-era flexibility statements as if they still solve the issue.
- Do not invent a UK base you do not truly operate from.
- Do not send a hybrid response that half-admits facts and half-argues aggressive legal absolutes.

3. Can you apply as an individual provider now, and what would that actually involve?

3.1 Yes, individuals can register

CQC says an individual can be the provider and should register as an individual if they will be carrying on the regulated activity themselves, i.e. as a sole trader.

If you are an individual provider and you are also in day-to-day charge, a separate registered manager may not be required. If you are not in day-to-day charge, a registered manager is required.

3.2 The present application route is not a neat online portal for new entrants

The current public route is a downloadable application-form process. CQC's current forms page provides the main new-provider registration form and an additional form specifically for online primary care services.

CQC's supporting-documents guidance says applications will be rejected if required documents are missing, incorrect, out of date, or irrelevant.

So, in practical terms, this is a form-and-evidence submission process, not a lightweight sign-up page.

3.3 Cost

The good news: CQC's fees guidance says there is no initial registration fee.

The catch: once registered, annual provider fees apply. CQC says the annual fee depends on the type and size of service, and the exact figure depends on service type selection and other registration details.

I did not find an official current webpage that cleanly states a single fixed annual fee for a small individual online-primary-care provider. CQC says it will invoice the exact amount and points providers to service-type guidance and fees guidance.

3.4 Non-negotiable application components

- Enhanced DBS with appropriate barred-list information before submitting the application.
- Main provider application form.
- Additional online primary care form.
- Supporting documents relevant to the service.
- Premises and information-governance arrangements ready enough for CQC to assess, and premises ready for a site visit if needed.

3.5 The premises/location trap

This is the nastiest practical issue. CQC's current location guidance says that if you are based outside England but are carrying on regulated activity in England, you will be expected to supply an address in England.

The same guidance says a PO box is not a location, and a virtual office is not a location because CQC must be able to inspect premises where regulated activities are carried on at or from.

For online primary care services, the location can be the headquarters or business address from which the service is delivered or managed. That makes a genuine business/control address important.

I did not find current official wording that positively endorses a Turkey-only address as the sole location for a provider carrying on regulated activity in England. The official guidance I found points the other way.

Bottom-line view:

- An immediate personal CQC application is legally possible in principle, but operationally awkward and strategically risky if the address model is weak.
- A Turkey-only address does not look like a published clean route.
- A fake, borrowed, or purely postal UK address would be worse than no application at all.
- If you apply, the address and operating model must be real, defensible, and inspection-ready.

4. Would home visits as an independent doctor solve this?

No. Home visits are not a safe workaround.

CQC's current independent-doctor guidance expressly includes private GP services and medical agencies that visit people in their home or other places where they are staying, such as hotels or care homes.

CQC's exemption guidance says the medical practitioner exemption does not apply to treatment provided outside a surgery or consulting room, and does not apply to treatment provided remotely by telephone, video consultation or email.

Location guidance says that where the provider travels to a variety of places to provide care, the location can be the address from which services are managed, which may be a headquarters or business address. That solves the "what is the location?" question, not the "do I need registration?" question.

Practical consequence:

If you did UK home visits as the person directing and controlling TDDI, you would still be walking into registration territory. Home visits change the logistics. They do not magically restore the independent-doctor exemption.

5. The Louise pathway: how this could work, and where people fool themselves

5.1 The core legal question is not "whose premises?" but "who is the provider?"

CQC's registration guidance repeatedly focuses on the legal entity carrying on the regulated activity, meaning the person or entity directing and controlling it.

Its additional guidance on arrangements between providers says the body holding ultimate responsibility must be the registered person for registration, compliance and enforcement purposes.

So simply seeing patients from Louise's clinic does not, by itself, hand your registration problem to Louise. If you remain the real provider, you may still need your own registration.

5.2 What Louise would actually be "buying" if she goes CQC

A lawful platform to run regulated private medical services rather than an informal clinic-plus-doctor arrangement.

Eligibility to be publicly found and assessed as a registered service, with inspection reports and ratings visible on the CQC website.

A structure that is easier to scale into broader primary care, weight management, minor procedures, remote follow-up, partnerships, and a deeper governance story with pharmacies and collaborators.

But also a governance burden: policies, complaints, records, consent, medicines governance, staffing, oversight, audits, risk management, and inspection exposure.

5.3 How to pitch it to Louise without sounding like a man setting fire to her weekend

Do not frame CQC as "please buy this so my problem goes away." She will smell the smoke immediately.

Frame it as: if the clinic wants to expand into a credible, scalable private medical model, CQC is the operating infrastructure that makes that possible.

Stress upside: broader service line, stronger patient trust, better partnership optics, easier recruitment of other clinicians, and a properly defensible brand.

Stress honesty: this is not just a certificate. It changes operations, records, accountability, and inspection readiness.

5.4 The red-line issue if Louise becomes the provider

The contracts, records, governance, billing logic, complaints handling, and clinical accountability all need to align with Louise's entity truly being the provider if that is the chosen model.

If the paperwork says one thing while the real-world control sits with you, CQC will not be impressed. Regulators enjoy substance over theatre.

6. Strategic options ranked for survivability

Option	Regulatory risk	Commercial upside	My view
Stop and respond under Option 2 only	Lowest immediate risk	Low short-term upside	Strongest immediate mitigation posture
Apply now as individual provider with a fully real UK operating address and full pack	Medium to high	Potentially high	Possible in principle, but not a panic move
Try Turkey-only premises model	High	Unclear	Weak published footing; avoid as a rushed gambit
Use home visits as workaround without registration	High	Low	Not a loophole
Louise clinic becomes properly registered provider, then redesign the model	Medium now, stronger later	High	Probably the best scalable structure if Louise is genuinely in
Work only under an already-registered provider's structure	Lower	Moderate	Operationally cleaner than solo improvisation

7. Your 28-day action plan

Days 1-3

- Preserve evidence of cessation: website changes, disabled bookings, paused ads, pharmacy/platform shutdown steps, any patient-facing notice if used.
- Build a chronology: when the service started, what services were offered, where you were physically located, where patients were located, which platforms were used, and when each activity stopped.
- Engage a healthcare regulatory solicitor and send them the enforcement letter plus your chronology.

Days 4-7

- Map every statement you may need to evidence. If you say "ceased", know exactly what ceased and when.
- Collect the official guidance passages that support misunderstanding rather than exemption certainty.
- Do not restart any UK-facing regulated activity while "thinking it through".

Week 2

- Decide, with your lawyer, whether the response is pure Option 2 with mitigation, or whether to add carefully framed contextual material about territorial misunderstanding.
- Separately, decide whether a future registration plan is realistic or just panic camouflage.

Week 3

- If exploring registration, pressure-test the address model, governance pack, DBS timing, and whether the provider should be you, a company, or Louise's entity.
- If exploring Louise, do not pitch "solve my problem"; pitch "build a proper platform".

Week 4

- Final lawyer-reviewed response to CQC before deadline.
- Final decision on future model: remain paused, apply properly, join another registered structure, or build Louise-led provider model.

Appendix A. Key source points reviewed

Health and Social Care Act 2008 (Regulated Activities) Regulations 2014, regulation 3: An activity is only a regulated activity if it is carried on in England.

CQC: Online primary care services (9 Mar 2026): CQC regulates providers of online primary care services in England where they are delivering a regulated activity by an online means.

CQC: What is a location? (30 Apr 2026): If you are a provider based outside England but carrying on regulated activity in England, you will be expected to supply an address in England. Virtual offices and PO boxes are not locations.

CQC: Choosing an online healthcare service (14 Apr 2026): CQC says it can only register services that are based in England.

CQC: Scope of registration, May 2022: CQC says it does not normally regulate providers based outside England that offer care and treatment to patients in England.

CQC: Medical practitioners in independent practice (29 Jan 2025): The independent-medical-practitioner exception does not apply outside a surgery or consulting room and does not apply to remote treatment by telephone, video consultation or email.

CQC: Independent doctor and clinic services (28 Nov 2025): This includes private GP services and medical agencies that visit people in their home, hotels or care homes.

CQC: Register as a provider / forms / supporting documents / DBS / fees: No initial registration fee; annual fees apply once registered; enhanced DBS required; online primary care requires an additional form; incomplete applications are rejected.

Appendix B. Source list

legislation.gov.uk/uksi/2014/2936/regulation/3/data.xht

cqc.org.uk/guidance-regulation/gps-online/online-primary-care-services-definition

cqc.org.uk/guidance-providers/registration/what-location

cqc.org.uk/care-services/help-choosing-care/choosing-online-healthcare-service

[cqc.org.uk/sites/default/files/2022-](https://cqc.org.uk/sites/default/files/2022-05/20220517%20Scope%20of%20Registration%20Guidance%20May%202022.pdf)

[05/20220517%20Scope%20of%20Registration%20Guidance%20May%202022.pdf](https://cqc.org.uk/sites/default/files/2022-05/20220517%20Scope%20of%20Registration%20Guidance%20May%202022.pdf)

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[registration/medical-practitioners-independent](https://cqc.org.uk/guidance-regulation/providers/registration/scope-registration/general-exceptions-and-exemptions-registration/medical-practitioners-independent)

cqc.org.uk/guidance-providers/independent-primary-medical/independent-doctor-clinic-services

[cqc.org.uk/guidance-regulation/registration/register-provider](https://www.cqc.org.uk/guidance-regulation/registration/register-provider)

[cqc.org.uk/guidance-regulation/providers/registration/register-provider/provider-application-forms](https://www.cqc.org.uk/guidance-regulation/providers/registration/register-provider/provider-application-forms)

[cqc.org.uk/guidance-regulation/registration/supporting-documents-provider](https://www.cqc.org.uk/guidance-regulation/registration/supporting-documents-provider)

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[cqc.org.uk/guidance-regulation/fees/provider-fees-payable](https://www.cqc.org.uk/guidance-regulation/fees/provider-fees-payable)